

Patient Name: John Doe
Age/Sex: 45/Male
Patient ID: 123456
Date of Examination: 01-Nov-2025

Referring Physician: Dr. Jane Smith

Chief Complaint:

Patient presents with persistent chest pain and shortness of breath for the past 3 days.

History of Present Illness:

Mr. Doe reports intermittent, pressure-like chest pain radiating to the left arm. Pain episodes last 10–15 minutes, exacerbated by exertion, relieved by rest. No associated syncope or palpitations. Denies fever, cough, or recent trauma. Past medical history includes hypertension and hyperlipidemia. No known drug allergies.

Physical Examination:

- **General:** Alert, oriented, in mild distress
- **Vital Signs:** BP 150/92 mmHg, HR 88 bpm, RR 18/min, Temp 98.6°F, SpO₂ 96%
- **Cardiovascular:** Regular rate and rhythm, no murmurs or gallops, peripheral pulses intact
- **Respiratory:** Clear breath sounds bilaterally, no wheezes or crackles
- **Abdomen:** Soft, non-tender, no organomegaly
- **Extremities:** No edema, pulses palpable

Investigations:

- ECG: ST-segment depression in leads V4–V6
- Chest X-ray: Normal cardiac silhouette, clear lung fields
- CBC & CMP: Within normal limits
- Troponin I: Elevated at 0.45 ng/mL (reference <0.04 ng/mL)

Assessment:

Acute coronary syndrome, likely NSTEMI.

Plan:

1. Admit to cardiology for monitoring and management.
2. Start antiplatelet therapy (Aspirin 81 mg daily, Clopidogrel 75 mg daily) and anticoagulation as per protocol.
3. Begin beta-blocker and statin therapy.
4. Cardiology consult for possible coronary angiography.
5. Monitor vital signs, cardiac enzymes, and ECG changes.
6. Educate patient regarding symptom monitoring and lifestyle modifications.

Physician Signature: _____

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